Pulmonary function in the cardiology clinic

Point-of-care complete PFTs with the MiniBox+

When a patient is short of breath, the heart is only half the picture. A complete pulmonary function test — performed in your own office in about 15–20 minutes — helps rule the lungs in or out on the same visit, with no referral and no delay. Every clinical figure below is drawn from the cited peer-reviewed sources and society guidelines and can be verified at the references listed.

Where PFTs change the cardiology workup

Sort cardiac vs. pulmonary dyspnea

Cardiac and pulmonary disease are the likeliest causes of breathlessness. Spirometry is a recommended **first-line** test in the chronic-dyspnea workup — the pulmonary half no longer waits on a referral.

Source: Chronic Dyspnea: Diagnosis and Evaluation, Am Fam Physician 2020;101(9):542–548.

Untangle heart failure and COPD

When spirometry is used to diagnose it, COPD is found in roughly **25–50%** of heart failure patients and is severely underdiagnosed. *Note: HF congestion can cause a reversible obstruction, so a PFT helps separate and co-manage — it is not a standalone COPD diagnosis.*

Sources: HF & COPD review (PMC12250576); airflow obstruction in stable systolic HF (PMC5219786).

Stratify peri-procedural risk

Abnormal pulmonary function is associated with more postoperative pulmonary complications after cardiac surgery. A PFT provides a baseline that helps **stratify risk** — a signal, not a pass/fail gate.

Sources: PFT in preoperative high-risk patients, Perioper Med 2024 (PMC10913451); cardiac-surgery spirometry (PMC7682508).

Monitor cardiotoxic therapy & drug-induced PH

The 2022 ESC cardio-oncology guidelines address monitoring for drug-induced pulmonary hypertension (notably dasatinib) — a targeted surveillance role for the right patients, not routine screening.

Source: 2022 ESC Guidelines on Cardio-Oncology, Eur Heart J 2022;43(41):4229–4361.

DLCO — the reason a spirometer isn't enough

A plain spirometer measures airflow only. A **complete PFT** adds lung volumes and **DLCO** (diffusing capacity), which reflects the gas-exchange surface and pulmonary vasculature — exactly where cardiac disease shows up in the lungs. In pulmonary hypertension due to **HFpEF**, nearly half of patients have a DLCO below 45% predicted, and reduced DLCO is associated with higher mortality.

Sources: Diffusion capacity & mortality in PH due to HFpEF, JACC: Heart Failure 2016; DLCO prognostic value in PH (PMC6935895).

What a complete PFT bills — in your office

CPT	Component	Nat'l avg
94010	Spirometry	$27.34
94726	Lung volumes (plethysmography)	$55.72
94729	DLCO (diffusing capacity)	$59.87
	Complete PFT, per patient	**~$151.45**

Add-ons when medically necessary: 94060 pre/post bronchodilator ($38.80) · 94375 flow-volume loop ($39.11). Billed under your practice — the study and the revenue stay in-house.

References (for verification): (1) Am Fam Physician 2020, Chronic Dyspnea — aafp.org/pubs/afp/issues/2020/0501/p542.html (2) PMC12250576 (3) PMC5219786 (4) PMC10913451 (5) PMC7682508 (6) JACC: Heart Failure 2016, DLCO & mortality in HFpEF-PH (7) PMC6935895 (8) 2022 ESC

Guidelines on Cardio-Oncology, Eur Heart J.

Billing & coding: CPT coding should be based on medical necessity; reimbursement is subject to specific health-plan policy. Figures are CMS national averages, consistent with current PulmOne toolkit rates, and are provided for informational discussion only. Device timing (~15–20 min complete PFT) reflects PulmOne product materials. This material is not a substitute for clinical judgment.